Clinical Study Summary Report (CSSR)
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1. Administrative & Study Identification

Full Study Title: Anticoagulation in Patients With Venous Thromboembolism and Cancer **Short Title/Acronym:** VICTORIE (VTE In Cancer—Treatment, Outcomes and Resource Use in Europe) **Protocol Number:** B0661150 **NCT Number:** NCT04618913 **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Sponsor/Company Name:** Pfizer **Investigational Product:** DOACs (Apixaban, Rivaroxaban, dabigatran etexilate [Trade Name: Pradaxa, ATC Code: B01AE07, EMA Link: https://www.ema.europa.eu/en/documents/product-information/pradaxa-epar-product-information_en.pdf], Edoxaban), VKA, LMWH **Phase of Development:** Not Applicable (Observational / Retrospective cohort study) **Medical Monitor:** Pfizer Study Management Team (N/A for retrospective claims study)

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To investigate treatment patterns, healthcare resource utilisation (HCRU), direct and indirect costs (where feasible), and safety and effectiveness outcomes in patients with Venous Thromboembolism (VTE) and active cancer, or patients with VTE and a history of cancer, who initiate anticoagulant treatment with a VKA, LMWH or NOACs. **Secondary Objectives:** To evaluate the comparative effectiveness and safety of Direct-Acting Oral Anticoagulants (DOACs) against standard Low-Molecular-Weight Heparins (LMWHs) or Vitamin K Antagonists (VKAs) in a real-world clinical setting.

2.2 Background & Rationale Venous thromboembolism (VTE) is a major complication in cancer patients, who have a significantly higher risk of VTE than the general population. While LMWHs have traditionally been the recommended standard of care, DOACs are increasingly replacing VKAs and serving as alternatives to LMWHs. This retrospective study generates real-world evidence using administrative and claims data across Europe to demonstrate the clinical utilization, economic impact, and actual safety/effectiveness profiles of these anticoagulant classes in oncology patients.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Retrospective cohort analysis) **Control Type:** Active Comparator (comparing different anticoagulant regimens) **Blinding:** Open-label (Database analysis) **Randomization:** N/A (Observational)

3.2 Planned Enrollment & Duration Target N: 1 **Number of Sites:** Multiple (European registry databases) **Estimated Study Start:** 14-Dec-2020 **Estimated Primary Completion:** 06-Apr-2026

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. A VTE diagnosis. 2. Active cancer or history of cancer. 3. Treatment with VKA, NOAC (apixaban, rivaroxaban, dabigatran, edoxaban) or LMWH. 4. Age ≥ 18 years at the date of index VTE (Venous thromboembolic).

4.2 Exclusion Criteria 1. Prior VTE diagnosis. 2. Diagnosis of prior atrial fibrillation. 3. Inferior Vena Cava (IVC) filter. 4. Prior exposure to oral anticoagulation (OAC) or parenteral anticoagulation (PAC) - note: Prophylactic use of OAC/PAC allowed. 5. Pregnancy. 6. More than one oral anticoagulation (OAC) or parenteral anticoagulation (PAC) dispensed on the index date. 7. Patients with less than one day of follow up. 8. Missing or incomplete data in the administrative/claims registry.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Real-world standard of care dosing as prescribed by the treating physician. **Route of Administration:** Oral (DOACs, VKAs) or Subcutaneous (LMWHs). **Duration of Treatment:** Real-world exposure duration tracked longitudinally via prescription refills.

5.2 Concomitant & Prohibited Therapy Permitted: Prophylactic use of OAC/PAC allowed prior to the index VTE; standard concomitant anti-cancer therapies. **Prohibited:** Concurrent initiation of multiple active anticoagulant therapies on the index date.

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6. Study Timeline & Visit Schedule

(Note: As a retrospective observational study, there are no scheduled clinical visits. Timeline reflects data extraction milestones.)

Visit ID	Window (Days)	Key Activities/Procedures
Screening	N/A	Retrospective cohort identification via ICD/claims codes
Baseline	Index Date	VTE event and initiation of index anticoagulant
Interim	N/A	Longitudinal tracking of HCRU, bleeding, and recurrent VTE via registry data
End of Study	N/A	End of continuous enrollment or limit of data availability

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: 1. **Recurrence of Thromboembolism:** The follow-up period during which outcome events of interest will be identified will run from the day after the index date until the end of the data collection or death, whichever occurs first. As data are updated at different times and frequencies in the different study countries, the date of the end of data collection will be determined after the relevant permissions have been acquired from the respective authorities. 2. Treatment patterns, healthcare resource utilisation (HCRU), direct and indirect costs, and safety (bleeding events) and effectiveness outcomes. **Measurement Method:** Extraction and analysis of ICD diagnostic codes, procedure codes, and pharmacy dispensing claims.

7.2 Secondary & Exploratory Endpoints Comparative safety (major bleeding) and comparative effectiveness (recurrent VTE) between patients on DOACs, VKAs, and LMWHs.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Retrospectively assessed via ICD diagnosis codes for bleeding or other complications in the claims databases. **Serious Adverse Events (SAEs):** Evaluated via inpatient hospitalisation records in the registry. **Data Monitoring Committee (DMC):** N/A (Non-interventional retrospective study).

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All eligible patients meeting inclusion criteria in the target databases. **Sample Size Justification:** Non-interventional convenience sample based on the maximum available qualifying data in the registries. **Handling of Missing Data:** Standard imputation techniques for administrative data or exclusion of incomplete records.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: Conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and applicable European data privacy regulations (e.g., GDPR). **Monitoring Plan:** N/A for site visits; automated data quality and integrity checks performed on extracted datasets. **Audit Trail:** Maintained via secure database access logs and statistical programming audit trails.

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11. Study Identifiers & Governance

Primary ID: B0661150 **Registry Number:** NCT04618913 **Sponsor/Lead Organization:** Pfizer **Study Title:** Anticoagulation in Patients With Venous Thromboembolism and Cancer **Official Regulatory Title:** VICTORIE (VTE In Cancer—Treatment, Outcomes and Resource Use in Europe)

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12. Clinical Framework (VTE & Cancer Specific)

Primary Condition: Cancer Associated Venous Thromboembolism (Disease Areas: Neoplasms, Embolism) **Condition Definition (Embolism):** Blocking of a blood vessel by an embolus which can be a blood clot or other undissolved material in the blood stream. (Semantic Type: Pathologic Function | MeSH Code: D004617 | SNOMED ID: 55584005) **Diagnosis Threshold:** Documented VTE diagnosis via ICD codes **Medical Methodology:** Anticoagulant Therapy (DOACs, LMWHs, VKAs) **Optimization Target:** Prevention of recurrent VTE and minimization of bleeding events

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13. Study Procedures & Methodology

Management Pathway: Real-world clinical pathway for VTE in cancer **Education Protocol (HCP):** N/A (Retrospective observational study) **Education Protocol (Patient):** N/A (Retrospective observational study) **Quality Audit Mechanism:** Database extraction verification and statistical quality control

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14. Observation & Follow-Up Schedule

Total Duration: Variable (based on patient records up to April 2026 or end of data collection/death) **Onsite Visit Cadence:** N/A (Registry-based) **Remote Monitoring Frequency:** N/A (Registry-based) **Checklist Review Interval:** Continuous retrospective tracking

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	**[02-Apr-2026]**
Clinical Operations Lead	**[Name]**	___	***[02-Apr-2026]***
Lead Statistician	***[Name]***	_____	[02-Apr-2026]